Pivotum

THE DEGREE RISK INDEX · FALL 2026 EDITION

So you're thinking about medicine

The short version — written for you, not about you.

Everything that matters, none of the bits written for your parents.

We scored twenty-seven careers on how exposed they are to AI. Medicine came out near the bottom of the risk range — meaning safe. Surgery scored 2.1 out of 10, the lowest score of anything we measured.

An entry-level software developer scored 8.1.

So if anyone has told you that tech is the sensible choice and medicine is the long slog, they've got half of that wrong.

The 60-second version

- Medicine is one of the safest careers there is. That part is settled.
- **But the specialties have stopped moving together.** Surgery 2.1. Emergency 2.5. Primary care 2.9. Radiology 4.9.
- You'll choose a specialty in five or six years, and for the first time that choice matters for this.
- The real problem with medicine isn't AI. It's that it takes ten to fifteen years.
- There's one skill nobody is teaching yet, and you'd be early to it.

Why it's protected

Four reasons, and none of them are about how clever AI gets.

You can't examine someone through a screen. Robotics is decades away from this, not product cycles away.

People want a human in the room when they're frightened. Nobody wants an algorithm to tell them they have cancer.

The law requires a licensed human — and licensing moves at the speed of politics, not technology. That makes it the most durable protection we score anywhere.

Patients don't match the textbook. They arrive with three things wrong at once, get worse unpredictably, and describe symptoms badly. That's exactly where AI is weakest.

The bit nobody tells you

Three years ago you could say "medicine is safe" and stop there. That's no longer a complete answer.

SPECIALTY	SCORE
Surgery	2.1
Emergency medicine	2.5
Primary care	2.9
Anaesthetics	3.1
Psychiatry	3.7
Pathology	4.7
Radiology	4.9

Same degree. Same training. A 2.8-point gap that gets wider every year we measure it.

The pattern is simple once you see it: **the protection concentrates where you're in the room with someone, and thins where the work is reading patterns off a screen.**

About radiology, since everyone asks

Our score says radiology is the most exposed specialty in medicine. Image reading is the single thing AI does best, and about three-quarters of all approved AI medical devices are in radiology.

The job market says the opposite. Around half of radiologist posts sit unfilled. Residency programmes match at close to 100%. Imaging demand keeps rising and the training pipeline is capped by funding.

Both are true. Being exposed to automation isn't the same as being unemployed, and radiology is where that gap is widest.

We publish the number with the disagreement attached rather than quietly adjusting it to look right. If the vacancy rates start falling, we'll change the score and say so.

What AI is actually doing to the job

Mostly taking the paperwork. Notes, letters, coding, looking up guidelines.

Here's the thing worth understanding, because it applies to every career you might consider: **AI goes after the routine, repeatable part of a job first.** That's true everywhere. What changes is whether the repeatable part was the bit people liked.

For a graphic designer, the repeatable part *was* the craft. So they end up checking a machine's version of their own work.

In medicine the repeatable part is documentation — which is the single biggest cause of doctors burning out. Automating it gives time back to patients.

That's an unusually good deal, and it's worth knowing it's not the norm.

The skill nobody is teaching

Knowing when the AI is wrong.

When a generated note doesn't match the patient. When a first-pass scan read is confidently, plausibly incorrect. That's a clinical skill, not a technical one — you can only catch a convincing error if you know the patient well enough.

Which means the boring foundational years matter *more*, not less.

And here's your opening. AI is already in most hospitals. It's arriving faster than the training to use it, and most working doctors are figuring it out alone.

So you'd arrive as part of the first group who never had to unlearn anything. In a profession where getting ahead normally takes years of experience, this is one dimension where being new is an advantage.

It won't make you better than a doctor with twenty years on a ward. It'll make you useful faster, and visible to the people who run things.

One caution, because school will have told you the opposite. Using AI to skip the work leaves you weaker — and in medicine that eventually becomes a patient-safety problem rather than a grades problem. But *understanding* AI is a completely different thing. That isn't cheating and never was.

The part we won't soften

It's ten to fifteen years to independent practice. Not four.

Sit with that properly. Where you'd be at 25. At 30. At 35. Compared with people you're at school with now who'll be established, earning and settled while you're still rotating through training posts and sitting exams.

Add: relocating repeatedly for jobs, and demanding hours through the years a lot of people are starting families. Burnout is real — around 42% of doctors report at least one symptom — though it has actually been falling for three years running, which is the opposite of what most coverage suggests.

And the thing doctors actually cite when they leave isn't the medicine. It's not being able to give the care you know someone needs, because of time or staffing. That has a name — moral injury — and it's different from being tired. It's what makes people walk away from a job they still believe in.

None of that is a reason not to do it. It's a reason to know what you're signing up for.

Does this actually sound like you?

Honestly, not everyone should do this. It tends to suit people who:

- Can decide well **without complete information**
- Get *calmer* when things go wrong, not more scattered
- Are genuinely interested in **people**, not only in biology
- Can wait a long time for the payoff

It's harder if you need control over your own schedule, need certainty and closure, or — and this is the big one — if you're mostly being pulled along by status or someone else's expectation. That last group is heavily over-represented among people who drop out.

Being squeamish, for the record, is almost never the problem people expect.

If you change your mind

You're not stuck. A medical degree opens onto research, public health, policy, health informatics, medical leadership, industry and expert work.

One catch, and it's the same pattern as everything else here: **the further a job sits from the patient, the higher its exposure climbs.** Those are all good careers. Just move deliberately rather than by drift.

What actually matters when picking a school

More than the ranking:

- **How early and how often you see real patients**
- Whether they teach you to use AI tools *and* spot when they're wrong
- Whether exams test **reasoning or recall** — recall is the automatable part
- What proportion match into their first-choice specialty
- How many students don't finish, and why

Two things worth doing

Find a doctor and ask them properly. Not a family friend who'll be encouraging — someone who'll be honest about the bad parts. Ask: *what are the new doctors on your team doing, and how is that different from when you started*? Half an hour with someone straight beats a term of careers advice.

Then ask yourself one question. Do you want *medicine*, or do you want what medicine is made of?

People are drawn to it for different reasons — helping people, the science, high-pressure problem-solving, wanting work that obviously matters, and sometimes the status. Those point in genuinely different directions, and some of them are reachable in four years instead of fourteen.

Worth knowing which one is yours before you commit a decade to finding out.

If you want to go further — three things worth twenty minutes

- Radiology offers more positions than ever — read this one first. It's the evidence that argues against our own score, and it's a good lesson in what it looks like when a prediction and a job market disagree.

- **Radiology in 2026: workforce crisis meets AI** — two forces hitting one specialty at once. Useful for seeing that pressures on a career are rarely just technological.
- **How AI will shape health care in 2026** — where these tools are actually landing in hospitals right now, and which specialties see it first.

There's a longer version behind this — full scores by specialty, how they were worked out, the honest downsides, what to ask a medical school, and where we might be wrong. Your parents have it.